2.16 Enteral Tube Feeding*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say he coughs during meals **Objective** • Coughs and chokes on food and drink • Failed the swallow screen	**ACTUAL** Impaired swallowing related to neuromuscular impairment as evidenced by coughing and choking during oral intake.	• Adequately nourished by tube without aspiration.	• Keep him nil by mouth until swallowing has been assessed. • Confirm tube position before each use, per policy. • Run the feed at the prescribed rate. • Refer to the speech and language therapist.	• Outcome reached when the feed is tolerated and weight holds steady.
Subjective • Complains of abdominal cramps **Objective** • Four loose stools a day • Tube feeding began 2 days ago	**ACTUAL** Impaired gastrointestinal motility related to enteral feeding intolerance as evidenced by frequent loose stools.	• Formed stools within 48 hours.	• Review the rate and strength of the feed. • Keep feeding sets clean and change them as policy states. • Go through his medications with the physician. • Track fluid balance.	• Considered met when stools are formed.
Subjective *No specific complaint* **Objective** • Nasogastric feeding • Less alert than usual	**RISK** Risk for aspiration as evidenced by risk factors: enteral feeding and decreased level of consciousness.	• No aspiration.	• Keep the head of the bed at 30–45 degrees during feeding and for an hour afterward. • Check tube position. • Measure gastric residual volume if ordered. • Stop the feed if he vomits or coughs.	• Goal reached if chest sounds stay clear.

* **References for this care plan:** [17], [35], [37], [55]. Full entries are listed in the References section of the CPMN book.